False, indisputable

False, because Gie City lied/withheld information

Government of South Australia
SA Health
Central Adelaide Local Health Network

16 MAR 2017

False/inaccurate, disputable

Subjective - projecting their values onto Gie City's life

Eastern Community Mental Health

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Date – 15/3/17

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
TUSMORE 5065

Dear Dr. Wong,

RE: Giles Culity, DOB 22/7/1993

I reviewed Giles in the Eastern Community Mental Health medical clinic today. Please find attached a summary and plan.

Today's review was organised after patient DNA usual clozapine clinic with Dr Ingole

Giles reported being well recently. He has noticed hypersalivation for which he is trialling atropine drops with some effect, continues to be bothered by it however. He has also had an increase in appetite which he is managing by watching intake. He had an episode of chest pain ~ 3 weeks ago with nil further, encouraged to go to ED if it recurs.
Giles reports his mood has been 'stable', denies feeling depressed. He reported ongoing suicidal thoughts - 'I would do it if I have to stay on medications for the rest of my life...' 'I have hope that I'll get off medications'. Was somewhat vague regarding any plans, stating 'I don't want to talk about it'. Giles was concrete regarding this, believing that his life would not be worth living if he had to stay on medications forever, but currently has hope that he will be off them in the future. He finds that his 'thinking is clouded', which has been a problem with 'all antipsychotics'.
Denied AH/VH. Reports that other doctors have told him he has 'delusions', but he does not agree. Thinks that he has 'androgen insufficiency syndrome', and has seen an endocrinologist who has offered testing which would cost $900 - Giles is contemplating this.
Sleeping 12 hours/night but feels well rested, energy levels good. Reports compliance with medications.

Meds
Clozapine 250 mg nocte
Aripiprazole 300 mg 4 weekly
Citalopram 30 mg daily

MSE
Casually dressed male of tall build, adequate self care. Polite and cooperative. Nil psychomotor agitation/retardation. Speech monotonous, themes of longstanding passive suicidal ideation, concrete in answers. Affect blunted, mood euthymic. Nil evidence of internal stimuli. Cognition NFA. Insight limited - does not believe has mental illness. Judgement variable - agrees to medications currently but not long-term.

Patient d/w Agnes, clozapine coordinator, and CC, Inge. Suicidal ideation ongoing and chronic, similar to baseline presentation. Nil recent concerns regarding mental state change.

Impression: schizoaffective disorder, chronic suicidal ideation with concrete thinking and likely chronic risk of self harm/suicide.

Plan
1. Continue current medications - await CBE and clozapine level from today
2. Continue with weekly reviews in clozapine clinic
3. Giles aware he can call ECMHS if concerned/questions

Please do not hesitate to call me if there are any questions or concerns.

Regards

Dr M Cavaiuolo
Psychiatry RMO
ECMHS
7425 5555